Case study: Chloe

Condition: **Spinal cord injury – T4 AIS B paraplegia**

26-year-old woman · Inpatient spinal rehabilitation unit, public hospital

Chloe Bennett is a 26-year-old woman with **T4 AIS B paraplegia** following a car crash seven weeks ago. She is a participant in the National Injury Insurance Scheme Queensland (NIISQ). Before her injury, Chloe worked as a primary school teacher's aide and played netball. She lives with her partner, Josh, in a two-storey townhouse in Brisbane's northern suburbs. She has been on the spinal rehabilitation unit for three weeks.

Chloe is medically stable, and physiotherapy reports good progress with sitting balance and wheelchair skills. Nursing notes over the past week describe her as "quiet but settling in". The team's notes record that she has missed four therapy sessions in the past week, mostly documented as "declined – tired".

You have been asked to complete an occupational therapy session on upper-body dressing and showering in Chloe's room. When you arrive, Chloe is in bed with the blinds closed and her breakfast tray untouched. Her phone lies face down on the bedside table, showing a string of unread messages from her netball team. She smiles politely and says she is "fine, just tired", and agrees to the session. She transfers to her wheelchair and puts on a T-shirt with minimal assistance, although she does not speak unless you ask her a question and keeps her eyes on the floor.

While you set up the bathroom, you see a note in the communication book on her bedside table. It is a message from Josh to the team, dated last Sunday, asking whether someone could talk to them about sexuality and fertility after spinal injury. Underneath, in different handwriting, someone has written: "Josh says C told him she'd 'rather be dead than live like this'. ?Pass on." You cannot find any documentation that this was followed up.

As you prepare to practise getting in and out of the shower, Chloe pulls her toiletry bag onto her lap and holds it tightly. When you ask whether she needs anything from it, she is silent for a moment. Then, quietly, she says, "I've been keeping my night-time tablets in here. I just pretend to swallow them. I've got enough now." She looks up at you. "Please don't tell anyone. You're the only one I've told. I trust you."

When you gently ask what she means by "enough", Chloe looks away and says, "Forget I said anything. I'm just tired." She asks whether you could "just leave the shower for today".

There is no documented mental health or risk assessment since Chloe arrived on the unit, and it is unclear whether she has been seen by psychology or social work. There is no record of how many tablets may be in the bag or what they are.

As you are speaking, there is a knock at the door. The recreation officer comes in with a flyer for a wheelchair sports come-and-try day tomorrow and says, "Netball girl, you'll love this!" Chloe smiles brightly and thanks her. A nurse then pops her head in to say Chloe's mum has called and will be here at 11 with photos of the townhouse for the home modifications, and that the NIISQ forms on the desk need Chloe's signature. Your next client is waiting in the gym. Chloe says, "Honestly, I'm fine. I'd just like to be on my own for a bit."

You are the occupational therapy student completing a dressing and showering session with Chloe and your clinical supervisor is not currently in the room.

Present Chloe's case to your clinical supervisor using the ISBAR framework. Identify the immediate concerns and explain what you would prioritise in responding to the situation.

ISBAR planning sheet

Use your 10 minutes of preparation to plan. Then speak for 7 minutes to your “supervisor”.

I Introduction

≈ 20 s

Who you are, your role, who the client is and why you need to talk now.

S Situation

≈ 40 s

A brief but complete summary of what is happening and the immediate concern.

B Background

≈ 1 min

Relevant history: the primary problem, medical and social history, medications/allergies, interventions so far and OT to date. Name missing information rather than assuming it.

A Assessment

≈ 2 min 20 s

Your interpretation. Case finding → what it may mean → consequence for safety and occupations → priority. Separate facts, interpretation, gaps and proposed assessment. What needs escalation?

R Recommendation

≈ 2 min 40 s

What should happen next, in priority order. Action → reason → who → review. Immediate safety, escalation, MDT input, one short-term and one longer-term OT assessment, intervention, referrals and follow-up.

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